

Clinical Study Summary Report (CSSR)

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1. Administrative & Study Identification

Full Study Title: A Phase III, Randomized, Double-Masked, Placebo-Controlled, Multicenter Study to Evaluate the Efficacy, Safety, Pharmacokinetics, and Pharmacodynamics of Satralizumab in Participants With Moderate-to-Severe Thyroid Eye Disease **Short Title/Acronym:** SatraGO-2 **Protocol Number:** GP44729 **NCT Number:** NCT06106828 **Trial Status:** Active, Not Recruiting **Sponsor Name:** Roche (Hoffmann-La Roche / F. Hoffmann-La Roche AG) **Investigational Product:** Satralizumab (Trade Name: Enspryng; ATC Code: L04AC19; Recombinant, humanized anti-interleukin-6 [IL-6] receptor monoclonal antibody) **Phase of Development:** Phase III **Medical Monitor:** Ethan Montenegro (Sponsor Representative / Clinical Operations)

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of satralizumab, specifically the proportion of participants with active, moderate-to-severe Thyroid Eye Disease (TED) who achieve a ≥ 2 -mm reduction in proptosis in the study eye from baseline at week 24. **Secondary Objectives:** To assess the safety, pharmacokinetics (PK), and pharmacodynamics (PD) of subcutaneous satralizumab, as well as its impact on proptosis change, diplopia, motility-induced pain, Clinical Activity Score (CAS), and participant quality of life.

2.2 Background & Rationale Thyroid Eye Disease (TED), also known as Graves Ophthalmopathy, is a rare, autoimmune, orbital inflammatory disorder that is disfiguring, debilitating, and potentially sight-threatening. There is a high unmet medical need for a fast-acting, durable, systemic disease-modifying therapy, as current options are associated with relapses and substantial side effects. Evidence supports an important role for IL-6 and its receptor in TED. Satralizumab binds to the IL-6 receptor, inhibiting IL-6 signaling and reducing proinflammatory and profibrotic signaling, offering a novel targeted therapeutic approach.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Placebo-controlled **Blinding:** Double-Masked (Double-blind) **Randomization:** Randomized (1:1 ratio to receive satralizumab or matching placebo, with a two-stage proptosis response-based re-randomization at week 24).

3.2 Planned Enrollment & Duration Target **\$N\$:** 127 participants (Actual enrollment: 127) **Number of Sites:** Multicenter (Global, including 4 EU countries, UK, USA, Canada, and others) **Estimated Study Start:** 15-Nov-2023 (Actual) **Estimated Primary Completion:** 24-Jul-2025 (Actual)

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years. 2. Clinical diagnosis of active, moderate-to-severe TED based on the Clinical Activity Score (CAS). 3. Appropriate candidate for systemic, disease-modifying therapy.

4.2 Exclusion Criteria 1. Decrease in CAS or proptosis of ≥ 2 points or ≥ 2 mm, respectively, in the study eye between Screening and Study Baseline (Day 1). 2. Requiring immediate surgical ophthalmological intervention or planning corrective surgery or irradiation during the course of the study, in the judgment of the investigator. 3. Identified pre-existing ophthalmic disease that, in the judgment of the investigator, would preclude study participation or complicate interpretation of study results, including corneal decompensation unresponsive to medical management and ophthalmic diseases that will likely require prohibited therapy. 4. Any serious medical condition or abnormality in clinical laboratory tests that, in the investigator's judgment, precludes an individual's safe participation in and completion of the study. 5. Pregnant or breastfeeding, or intention of becoming pregnant during the study or within 3 months after the final dose of satralizumab.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Administered every 4 weeks (Q4W). In Part I, participants receive satralizumab or placebo Q4W. In Part II, treatment is individualized based on week 24 proptosis response. **Route of Administration:** Subcutaneous (SC) injection. **Duration of Treatment:** 72 weeks of overall study participation (Part I: Weeks 1-24, Part II: Weeks 24-48, Follow-up: Weeks 48-72).

5.2 Concomitant & Prohibited Therapy Permitted: Standard medical management. Rescue intervention is permitted starting from week 4 based on protocol-defined criteria as assessed by the investigator. **Prohibited:** Pre-existing ophthalmic diseases that will likely require prohibited therapy, and concomitant use of treatments that confound systemic IL-6 blockade.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures
Screening	Day -28 to 0	Informed Consent, baseline CAS and proptosis measurements.
Baseline	Day 1	Randomization (1:1), First SC dose of satralizumab/placebo.
Part I	Weeks 1 to 24	Q4W dosing, safety tracking, proptosis and CAS efficacy evaluation.
Part II	Weeks 24 to 48	Proptosis response-based re-randomization, Q4W continued dosing.
Follow-up	Weeks 48 to 72	Off-treatment safety follow-up, final exit assessments.

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: The percentage of participants with active, moderate-to-severe TED who achieved a ≥ 2 -millimetres (mm) reduction in proptosis from baseline (Day 1) at Week 24 in the study eye, provided there is no deterioration of proptosis (≥ 2 mm increase) in the fellow eye. **Measurement Method:** Standardized clinical ophthalmological examination.

7.2 Secondary & Exploratory Endpoints

1. Change in proptosis.
2. Percentage of participants achieving ≥ 1 grade reduction/improvement in diplopia among participants with baseline diplopia.
3. Percentage of participants achieving absence of motility-induced pain.
4. Change from baseline in Clinical Activity Score (CAS).
5. Change from baseline in the Graves' Ophthalmopathy Quality of Life (GO-QoL) questionnaire.
6. Pharmacokinetics (PK) and Pharmacodynamics (PD) of satralizumab.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Routine collection via regular onsite/remote visits, analyzing AEs related to subcutaneous injection and systemic immunomodulation. **Serious Adverse Events (SAEs):** Reported per standard safety protocols. Treatment policy is applied to AE/efficacy-related discontinuations. **Data Monitoring Committee (DMC):** Utilizes Clinical Endpoint Committees (CEC) for confirmed protocol-defined relapses or clinical relapses.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) population for primary efficacy outcomes; Safety Set for all AE assessments. **Sample Size Justification:** $N=127$ participants, sufficiently powered to detect a statistically significant increase in the proportion of proptosis responders at Week 24 vs. placebo. **Handling of Missing Data:** Missing data handled via multiple

imputation (Missing At Random [MAR] assumption). A composite strategy is used for other intercurrent events, where data after intercurrent event handling is imputed as non-responder.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.
Monitoring Plan: Regular assessments to evaluate safety, CAS, and PK/PD. Strata are mapped accurately by current smoking status and TED activity status using an IxRS-based system. **Audit Trail:** Robust clinical data management including continuous safety and endpoint monitoring.

11. Study Identifiers & Governance

Primary ID: GP44729 **Registry Number:** NCT06106828 **Posting ID:** 952312382347868769 **Sponsor/Lead Organization:** Roche **Study Title:** SatraGO-2 **Official Regulatory Title:** A Phase III, Randomized, Double-Masked, Placebo-Controlled, Multicenter Study to Evaluate the Efficacy, Safety, Pharmacokinetics, and Pharmacodynamics of Satralizumab in Participants With Moderate-to-Severe Thyroid Eye Disease

12. Clinical Framework (Thyroid Eye Disease Specific)

Primary Condition: {{Thyroid Eye Disease (Graves Ophthalmopathy)}} **Diagnosis Threshold:** {{Active, moderate-to-severe disease based on CAS}} **Medical Methodology:** {{Interleukin-6 (IL-6) Receptor Blockade}} **Optimization Target:** {{25% reduction in proptosis}}

13. Study Procedures & Methodology

Management Pathway: {{Two-stage, proptosis response-based individualized SC treatment}} **Education Protocol (HCP):** {{Training on standardized proptosis measurement and CAS evaluations}} **Education Protocol (Patient):** {{Self-administered GO-QoL questionnaire & SC injection monitoring}} **Quality Audit Mechanism:** {{IxRS-based tracking of patient smoking status & disease activity}}

14. Observation & Follow-Up Schedule

Total Duration: {{72 weeks}} **Onsite Visit Cadence:** {{Screening, Baseline, Week 24, Week 48, Week 72}} **Remote Monitoring Frequency:** {{Regular intervals to capture AEs/QoL assessments}}

Checklist Review Interval: {{Every 4 weeks (Q4W) concurrent with dosing}}

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator	[Name]	_____	07-Apr-2026					
Clinical Operations Lead	[Name]	_____	07-Apr-2026					Lead
Statistician	[Name]	_____	07-Apr-2026					